From Reconstruction to Anticipation

Predicting future tissue geometry from monocular laparoscopic video

FYP-2 Mid Evaluation · Modules 5 and 6

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The laparoscope shows the present, never the next moment

A surgeon operates through one camera. It gives a flat image with no depth, no 3D structure, and no indication of how tissue will respond to the instrument about to touch it.

Reconstruction research has solved the first problem well. It has not addressed the second at all.

Every published surgical 4D reconstruction system is retrospective: it models deformation that has already been recorded.

What exists today
Reconstruct the scene in 3D, for frames already captured

What is missing
Anticipate where tissue will be a moment from now, with an honest indication of confidence

0	1–2 px	0	10
depth sensors available	tissue motion per frame	deformation labels in clinical video	videos reconstructed

WHERE WE ARE

FYP-1 built the reconstruction. FYP-2 adds the prediction

M1 Frames + masks → **M2** 4D Gaussian reconstruction → **M3** Trajectory extraction → **M4** Self-supervised windows → **M5** Diffusion predictor → **M6** Inference + evaluation

FYP-1 complete | FYP-1 complete | FYP-1 complete | FYP-1 complete | FYP-2 focus | FYP-2 focus

30.10 dB
reconstruction PSNR

0.904
reconstruction SSIM

57,943
Gaussians tracked

7,371
windows built (train + val)

Module 5 — learning how tissue moves

A conditional diffusion model that learns the deformation of 512 tracked tissue points, trained entirely without clinical labels.

Input
- Last 4 frames of 512 tracked points
- How far ahead to predict (1, 2 or 3)
- A patch of random noise to shape

Model
- 16.3 M parameters
- Temporal transformer per point
- Two-way cross-attention
- Denoises over 20 steps

Output
- Movement of each point
- Sample repeatedly → several plausible futures

Why diffusion and not direct regression
Soft tissue under contact is genuinely uncertain. A single estimate hides that. Sampling the model repeatedly gives a spread of outcomes — which Module 6 shows is a usable confidence signal.

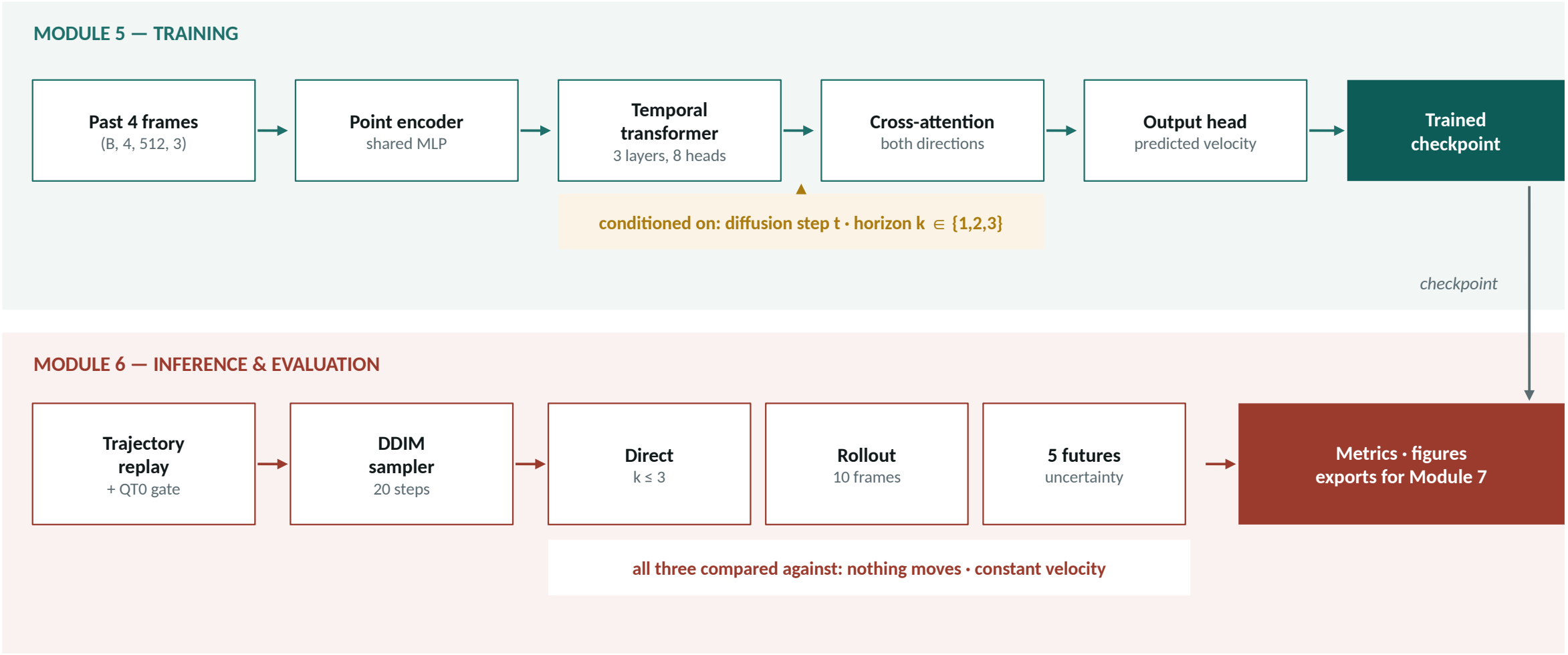
Why the horizon is an input
The same 4 frames appear in training with targets 1, 2 and 3 frames ahead — three different right answers. Without being told which, the task is unlearnable no matter the capacity.

Built from 7,371 windows across 10 reconstructed videos — 5,910 train, 1,461 validation — with no human annotation at any stage.

Module 5 — what went wrong, and what we did

The task had no single right answer One context appeared with targets 1, 2 and 3 frames ahead.	→	**Condition on the horizon** Horizon embedded and injected into both branches of the network.
The signal was smaller than the noise Tissue moves 0.004 units; diffusion adds noise at scale 1.	→	**Rescale the target to unit variance** Inverted before every reported metric, so results stay in physical units.
Raw trajectories had almost no momentum Correlation between consecutive motions was 0.077 — nearly noise.	→	**Adaptive temporal smoothing** Per-video Savitzky–Golay window; correlation rose to 0.89.
Training made the model worse after epoch 60 Held-out error rose while training loss kept falling.	→	**Shorter schedule, stronger regularisation** 500 → 200 epochs, weight decay 0.01 → 0.05, best checkpoint kept.
A model that gave up scored the same as doing nothing Predicting zero displacement equalled the naive baseline.	→	**Predict the correction to constant velocity** Now giving up falls back to an estimator 41% better than naive.

Architecture — Modules 5 and 6



Module 6 — using the model, and measuring it honestly

Module 6 trains nothing. It loads the trained model, asks it three different questions, and grades every answer against estimators that need no learning at all.

Ask once

Where will tissue be 1, 2 or 3 frames from now?

Most reliable mode.

Ask repeatedly

Feed each answer back in to reach 10 frames ahead.

Error compounds — measured.

Ask five times

Five possible futures from one input. How much they disagree is the confidence estimate.

Graded against two simple rules

- "Nothing moves" — the naive baseline
- "Everything continues straight" — constant velocity

If a 16 M-parameter model cannot beat these, we say so.

Six quality tests

QT0 verifies the evaluation data is the data the model trained on, before anything is predicted. QT1–QT5 cover accuracy, rollout, diversity, smoothness and cross-scene generalisation.

Module 6 is not there to make the model look good. It is there to establish, with evidence, exactly where the model stands.

PROBLEMS → SOLUTIONS

Module 6 — what went wrong, and what we did

Evaluation could have measured the wrong data Rollout needs continuous trajectories; Module 4 stores fixed windows.	→	**Rebuild, then verify before predicting** QT0 checks the rebuild against Module 4 output and runs as a gate.
The metric was penalising the model unfairly Chamfer Distance is minimised by the average, not by one sample.	→	**Average several sampled trajectories** Removes a ~40% penalty the model was being charged for free.
Diagnostics were measuring one easy video Taking the first N windows drew almost entirely from one recording.	→	**Evenly spread the evaluation set** Every video contributes; earlier scores had been 4× too optimistic.
Our constant-acceleration baseline was wrong A backward difference gives the velocity at t − ½, not at t.	→	**Derive the velocity at t properly** $v = (3 \cdot x(t) - 4 \cdot x(t-1) + x(t-2)) / 2$, verified exact on quadratic motion.
What we called a Kalman filter was not one It was an alpha–beta filter with fixed gains and no covariance.	→	**Implement a real two-state Kalman filter** Full 2×2 covariance; recovers a known velocity to 2.6e−06.

Training converged cleanly — and then overfitted

16.3 M
parameters

500
epochs completed

0.012
training loss, from 0.36

~60
best epoch

What went right

Smooth convergence, no instability, no divergence.
Multi-session training resumes exactly where it stopped,
surviving Colab disconnection.

What went wrong

Held-out error bottomed out near epoch 60 and rose after
~260 while training loss kept falling — overfitting,
measured directly rather than assumed.

Best held-out Chamfer Distance 0.01568 versus naive baseline 0.01557

The model converged to predicting almost no motion — which is exactly what the naive baseline does. Understanding why is the rest of this presentation.

A two-line formula beats our model

Method	k = 1	k = 2	k = 3
Nothing moves (naive)	0.0093	0.0165	0.0222
Constant velocity	**0.0055**	**0.0138**	**0.0187**
Ours — diffusion	**0.0093**	**0.0165**	**0.0225**

Chamfer Distance, lower is better · full held-out set, 10 videos

Read this honestly

Our model sits exactly on the naive baseline at every horizon.

Constant velocity beats both — by 41% at one frame ahead.

It loses on all 10 videos.

Rollout stays stable (growth exponent 0.61) and two explanations were tested and eliminated:

- Error does not grow with distance from training frames (ρ = +0.02, p = 0.61)
- Dropped preprocessing frames do not explain it (333 origins checked, 0 drops found)

The model knows which way tissue moves — not how far

Chamfer Distance alone cannot separate "learned nothing" from "learned the direction but the wrong distance". Measuring them separately can.

Agreement between predicted and true motion direction

t+1	t+2	t+3	t+5	t+10
+0.57	+0.40	+0.26	−0.01	−0.49

Why this matters

A model that learned nothing would read 0.00 everywhere.
+0.57 with naive-level error can only mean the magnitude is wrong — a far more tractable problem.

What follows from it

- Predict the correction to constant velocity (Module 5)
- Calibrate the magnitude at inference (Module 6)

Both implemented; evaluation is the next run.

The model knows when it should not be trusted

r = 0.69
spread vs actual error

5
futures sampled per input

5×
uncertainty growth over 10 frames

When the five sampled futures disagree, the prediction really is worse.

Uncertainty rises smoothly with how far ahead we predict — 0.00011 at one frame, 0.00055 at ten.

Why this is the result we are proudest of

A system that is confidently wrong is unusable in theatre.
A system that signals "do not trust me here" is usable even when imperfect.

No method in our comparison provides this.

Stated carefully

This is correlation between predicted spread and realised error. Some of it may reflect that busy moments are both harder and more variable — we say so in the paper rather than claiming more than we measured.

Making the prediction visible

A prediction moves 512 points. A picture needs all 58,000. Bridging that gap is what turns a metric into something a surgeon could look at.

From points to pixels

Predicted motion of the 512 tracked points is carried onto every Gaussian by distance-weighted interpolation, so a tracked point receives its own motion exactly.

The renderer, re-verified

Module 2's renderer was re-implemented here, including degree-3 spherical harmonics. We checked it against the original rather than assuming they matched.

Six panels per moment

Real frame, reconstruction ceiling, prediction, naive baseline, motion overlay and an amplified error map — with image metrics over the full frame and over tissue alone.

Predicted motion was invisible

At 1–2 pixels, predicted and real frames look identical. Motion is drawn as arrows coloured by agreement with truth, magnified automatically — and the factor is printed on every panel, never applied to any reported number.

Nothing on screen was a prediction

Every predicted frame used to be paired with a frame that already existed. The forecast now runs 12 frames past the end of the recording: the real-video panel stops and says so, while the predicted panel keeps going.

30.04
reconstruction ceiling, dB

26.79
our prediction, dB

26.74
naive baseline, dB

Four adjacent fields. None occupies the cell we need

Line of work	Representative	Predicts future?	Output space	Uncertainty
Surgical 4D reconstruction	EndoNeRF, LerPlane, EndoGaussian, Deform3DGS	No — retrospective	3D geometry	No
Surgical video anticipation	TPG-VAE, SurgVista	Yes	2D pixels only	No
Point cloud forecasting	PointRNN, MoNet, PCPNet	Yes	3D, rigid outdoor LiDAR	No
Medical point diffusion	Xie et al. 2025 (liver)	No — present state	3D points	No
This work	**—**	**Yes**	**3D tissue geometry**	**Calibrated**

The contribution is the combination in the last row, not any single column: future 3D tissue geometry, from monocular video, with no annotation, and a calibrated confidence estimate.

How we compare with published work

3D reconstruction — base paper family

Method	Input	PSNR
LerPlane (MICCAI 23)	stereo	29.46
EndoGaussian (24)	stereo	30.25
Deform3DGS (MICCAI 24)	stereo	30.48
Ours	**monocular**	**30.10**

Indicative, not controlled: published figures use stereo supervision on different sequences. We state this in the paper.

What the left table establishes

Our reconstruction sits within 0.4 dB of the stereo state of the art while using a single camera and no depth sensor.

That geometry is the input Module 5 learns to predict.

Prediction — base paper family

PointRNN (Fan & Yang, 2019)

The closest published method: future point clouds from past ones, scored with Chamfer Distance. Implemented faithfully as our primary learned baseline.

Luo & Hu (CVPR 2021)

First diffusion model over 3D point clouds. Our denoiser is a conditional, temporally-grounded instance of it.

Full comparison in progress

11 methods on one protocol: 5 classical, 5 learned, ours. Module 7 implemented; the run is the next step.

What comes next

1. **Evaluate the two fixes already implemented**
   Calibration needs no retraining and should move accuracy below naive.
2. **Retrain on the constant-velocity residual**
   Raises the model's floor from the naive baseline to a 41% stronger one.
3. **Hold out an entire video for testing**
   Removes the optimism of selecting and reporting on the same frames.
4. **Complete the 11-method comparison**
   Modules 7 and 8 are implemented; the runs remain.
5. **Grow the dataset beyond 10 short recordings**
   The overfitting we measured is consistent with corpus size.

Where Modules 5 and 6 stand

Delivered

- A complete prediction pipeline, no clinical labels
- A six-test evaluation framework with an integrity gate
- Calibrated uncertainty (r = 0.69)
- Comparative study implemented (11 methods)
- Conference paper drafted (IEEE format)

Not yet achieved

- Beating the naive baseline on accuracy

We localised the cause to displacement magnitude, not direction, and implemented two fixes that follow directly from that measurement. Evaluating them is the immediate next step.

An honest negative result with an identified cause is worth more than an unexplained positive one.